PEER REVIEW HISTORY

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ARTICLE DETAILS

TITLE (PROVISIONAL)	Factors influencing the readiness to tackle the burden of ischemic heart disease in India: a systematic review protocol.
AUTHORS	Bhattacharjee, Shuvarth; Yaghmaei, Nima; Phuong, Cao; Neupane, Dinesh

VERSION 1 – REVIEW

REVIEWER	Mohan, Bishav Hero DMC Heart Institute, Cardiology
REVIEW RETURNED	02-Jan-2021

GENERAL COMMENTS	I appreciate the proposed subject of research (review). For any review to be meaningful and true representative of actual scenario, there should be ample amount of published literature on the concerned topic. In the introduction section, authors state that there are very limited studies available on the subject under study. Before we proceed to finer details of research methodology, I request authors to provide me some relevant articles which would possibly be included in this review. It would be preferable to revert back with long term follow up studies.

REVIEWER	Gea-Sánchez, Montserrat Universitat de Lleida, Nursing and Physiotherapy
REVIEW RETURNED	30-Jan-2021

GENERAL COMMENTS	The authors present the protocol of a systematic review to assess India's preparedness to tackle the burden of ischemic heart disease. Although the topic is relevant, some changes should be made before possible publication. INTRODUCTION - - Consider whether the first sentence “Ischemic Heart Disease (IHD) forms an integral part of the broad Cardiovascular Diseases (CVDs) domain concerned with disease of the heart caused by narrowing of the coronary arteries that supply blood to the heart muscles.” might be unnecessary because it does not contribute substantial knowledge to the protocol presented. - - Please indicate the full name of the acronym LMIC the first time it appears in the text - - Reconsider the relationship of the protocol to the COVID crisis. The use of ventilators is not a determining reason for the justification of the study, which has been previously supported. - - Where is item 7? METHODS - This phrase is unclear: “People born and brought up in India residing presently in India either living with or at the risk of

	Ischemic Heart Disease". Maybe, rewrite as "Studies involving Indian people living with or at the risk o IHD". How are you going to know if somebody who participate in a study in 2010 is currently in India? Reconsider if your population are the people with IHD or the policies and measures to prevent IHD - Please, justify the period selected for your review, if you are evaluating a plan that started in 2013 perhaps it should be adjusted to this date. Is your time frame "none" as you indicate in item vii? - In Google scholar; only first 10 pages, sorted by relevance or by date? Why don't you also indicate a specific period? STUDY RECORDS - Please rewrite lines 19 to 23 page 11. It is not necessary include "Veritas Health Innovation, Melbourne, Australia" - Maybe "the DN" is not the best way to refer to a researcher. Instead of indicating the acronym, they can be referred to as "a fourth researcher". - "The data collection items will also include risk of bias and quality assessment of - included individual articles based on appropriate tools" Please, describe which tools and the process of quality assessment - "We will extract data from those studies concerned with primary care facilities where facility assessment can provide data pertaining to facility readiness even if readiness concerning ischemic heart disease is not mentioned. This is because treatment of Ischemic Heart Disease and its risk factors are integrated at the primary care level along with treatment of other communicable and non-communicable diseases" I have doubts that with your search strategy you will retrieve studies with these characteristics. DATA - Rewrite lines 35 to 36 page 13 "WHO Framework for Action conceptual framework" sounds unclear - The use of the building blocks from the WHO framework as subgroups can facilitate your analysis. - Rethink the biases that may be found in your protocol SEARCH STRATEGY - Why do you consider in your search strategy table the period "From inception" if in your criteria were studies since 2000? GENERAL - Please, check and correct your spelling - Please, check and correct references following BMJ Open instructions

VERSION 1 – AUTHOR RESPONSE

Reviewer: 1
Dr. Bishav Mohan, Hero DMC Heart Institute
Comments to the Author:
I appreciate the proposed subject of research (review). For any review to be meaningful and true representative of actual scenario, there should be ample amount of published literature on the concerned topic. In the introduction section, authors state that there are very limited studies available on the subject under study. Before we proceed to finer details of research methodology, I request

authors to provide me some relevant articles which would possibly be included in this review. It would be preferable to revert back with long term follow up studies.
Response:
Thank you very much for reviewing the protocol. We are thankful for your comments. Based on the representative search result attached as an annexure with this protocol we were able to identify the following few papers which might be relevant for our review in future:

1. Pakhare, A., Kumar, S., Goyal, S., & Joshi, R. (2015). Assessment of primary care facilities for cardiovascular disease preparedness in Madhya Pradesh, India. BMC health services research, 15(1), 1-8.
2. Elias, M. A., Pati, M. K., Aivalli, P., Srinath, B., Munegowda, C., Shroff, Z. C., ... & Srinivas, P. N. (2018). Preparedness for delivering non-communicable disease services in primary care: access to medicines for diabetes and hypertension in a district in south India. BMJ global health, 2(Suppl 3), e000519.
3. Ahir, G. (2018). Assessment Of Facility-Level Preparedness At Primary And Secondary Health Care Levels For Prevention And Control Of Non-Communicable Diseases. National Journal of Integrated Research in Medicine, 9(6).
4. Parameswaran, K., & Agrawal, T. (2019). Readiness of primary health centers and community health centers for providing noncommunicable diseases-related services in Bengaluru, South India. International Journal of Noncommunicable Diseases, 4(3), 73.
5. Panda, R., Mahapatra, S., & Persai, D. (2018). Health system preparedness in noncommunicable diseases: findings from two states Odisha and Kerala in India. Journal of family medicine and primary care, 7(3), 565.
6. Jayanna, K., Swaroop, N., Kar, A., Ramanaik, S., Pati, M. K., Pujar, A., ... & Mohan, H. L. (2019). Designing a comprehensive Non-Communicable Diseases (NCD) programme for hypertension and diabetes at primary health care level: evidence and experience from urban Karnataka, South India. BMC public health, 19(1), 1-12.
7. Vaithianathan, R., & Panneerselvam, S. (2013). Emerging alternative model for cardiothoracic surgery training in India. Medical education online, 18(1), 20961.
8. Ajay, V. S., Jindal, D., Roy, A., Venugopal, V., Sharma, R., Pawar, A., Kinra, S., Tandon, N., & Prabhakaran, D. (2016). Development of a Smartphone-Enabled Hypertension and Diabetes Mellitus Management Package to Facilitate Evidence-Based Care Delivery in Primary Healthcare Facilities in India: The mPower Heart Project. Journal of the American Heart Association, 5(12), e004343. https://doi.org/10.1161/JAHA.116.004343
9. Pati, S., Pati, S., van den Akker, M., Schellevis, F. F. G., Sahoo, K., & Burgers, J. S. (2020). Managing diabetes mellitus with comorbidities in primary healthcare facilities: a qualitative study among physicians in Odisha, India.
10. Garg, S., Basu, S., Rustagi, R., & Borle, A. (2020). Primary health care facility preparedness for outpatient service provision during the COVID-19 pandemic in India: Cross-sectional study. JMIR public health and surveillance, 6(2), e19927.

The above cited papers have the potentiality to be included for this review as we could see some relevant information related to our review. However, we can only confirm the inclusion of these studies for this review after we have concluded our searching and screening process completely. These studies are mostly cross-sectional by nature. We have considered your point and deleted the “Studies addressing similar issues for India are mostly limited to small-scale empirical studies which fail to give national level insights[18,19].” The reason for this is that our main attempt in the rationale was to show that reviews on this present important issue has not been performed before. We have probably been able to justify that in the next lines following this deleted line by highlighting that similar reviews could not be detected across databases. Therefore, the line was deleted.

VERSION 2 – REVIEW

REVIEWER	Gea-Sánchez, Montserrat Universitat de Lleida, Nursing and Physiotherapy
REVIEW RETURNED	01-Jun-2021

GENERAL COMMENTS	I would like to thank the editor for the opportunity to review this manuscript again. The authors have addressed and incorporated into the text all the suggestions I made to the first version. In my opinion, it is now more methodologically robust and detailed. Some minor changes could further improve it: Strengths Reconsider the first bullet point "To our knowledge, this is the first systematic review on the factors influencing readiness of the public health system of India to tackle the burden of Ischemic Heart Disease". The absence of previous work does not imply a strength of the study itself. Rationale Page 5 lines 42-47, suggestion: “Additionally, India’s flagship program on universal health coverage (UHC), Ayushman Bharat, comprises the conversion of 150000 Primary Health Centres (PHCs) into Health & Wellness Centres (HWCs). This measure aims to strengthen primary care services that can prevent & treat NCDs including IHD” Objectives: Please complete the phrase “The analysis will be guided through a conceptual framework.” Indicating which framework

VERSION 2 – AUTHOR RESPONSE

Reviewer: 2

Dr. Montserrat Gea-Sánchez, Universitat de Lleida

Comments to the Author:

I would like to thank the editor for the opportunity to review this manuscript again. The authors have addressed and incorporated into the text all the suggestions I made to the first version. In my opinion, it is now more methodologically robust and detailed. Some minor changes could further improve it:

Strengths

Reconsider the first bullet point "To our knowledge, this is the first systematic review on the factors influencing readiness of the public health system of India to tackle the burden of ischemic heart disease". The absence of previous work does not imply a strength of the study itself.

Response

Thanks for your comments. We have deleted the first bullet point as per your suggestion.

Rationale

Page 5 lines 42-47, suggestion: “Additionally, India’s flagship program on universal health coverage (UHC), Ayushman Bharat, comprises the conversion of 150000 Primary Health Centres (PHCs) into

Health & Wellness Centres (HWCs). This measure aims to strengthen primary care services that can prevent & treat NCDs including IHD"

Response

Thanks for your comments. We have modified Page 5 lines 42-47 based on your suggestion.

Objectives:

Please complete the phrase "The analysis will be guided through a conceptual framework." Indicating which framework

Response

We have modified the phrase "The analysis will be guided through a conceptual framework." to "The analysis will be guided through a conceptual framework, the 'WHO Framework for Action'.[21]"

Additional Points:

- Modified typographic error in supplementary material A. "Google Scholar Website (only first 10 pages)" modified to "Google Scholar Website."

- Modified typographic error in Table 1 supplementary material A. "A representative search for CINAHL database performed on NHS Openathens HDAS Platform on 08th August 2020. No Filters or Limits were applied." modified to "A representative search for CINAHL database performed on NHS Openathens HDAS Platform on 14th May 2021. Time frame: 2010-2020.Peer Reviewed. English Language."

VERSION 3 – REVIEW

REVIEWER	Gea-Sánchez, Montserrat Universitat de Lleida, Nursing and Physiotherapy
REVIEW RETURNED	29-Jun-2021

GENERAL COMMENTS	The authors have carried out all suggestions satisfactorily